



January 27, 2025

The Honorable Kathy Hochul
Governor of New York State
NYS State Capitol Building
Albany, NY 12224

RE: Letter in Opposition to the New York Health Information Privacy Act (AB 2141 and SB 929)

Dear Governor Hochul:

On behalf of the advertising industry, we request that you veto or seek amendments to the New York Health Information Privacy Act under AB 2141 and SB 929 (collectively, “NYHIPA” or “the bill”).¹ The bill would hinder New Yorkers’ ability to access services and severely burden the operation of many businesses in the state. We provide this letter to express our non-exhaustive list of concerns about NYHIPA. Our organizations support the enactment of meaningful privacy protections for New Yorkers. However, as engrossed, NYHIPA would have far-reaching, unintended, and unfavorable consequences for New York consumers and the business community alike.

As the nation’s leading advertising and marketing trade associations, we collectively represent thousands of companies across the country. These companies range from small businesses to household brands, publishers – large and small, advertising agencies, and technology providers. Our combined membership includes more than 2,500 companies that power the commercial Internet, which accounted for 12 percent of total U.S. gross domestic product (“GDP”) in 2020. Our group has more than a decade’s worth of hands-on experience it can bring to bear on matters related to consumer privacy and controls. We would welcome the opportunity to engage with you further to discuss the issues we catalog in this letter.

I. NYHIPA’s Definitions of “Regulated Health Information” and “Regulated Entity” are Overly Broad

NYHIPA’s terms, coupled with its overly broad definition of “regulated health information,” could unintentionally impede New Yorkers from receiving useful and relevant information about products and services they may desire. As defined, the term “regulated health information” would include *any information* that could possibly be related—however tangentially—to the health of a consumer.² The definition could be interpreted to include basic data points, such as the fact that a consumer purchased non-prescription shampoo for individuals with dry hair at a local grocer, attended a fitness event, or signed up to receive promotional notices about specific clothing or

¹ New York AB 2141 (2025 Sess.), located [here](#); New York SB 929 (2025 Sess.), located [here](#) (collectively hereinafter, “NYHIPA”).

² *Id.* at § 1120(2).

footwear restocks. None of this information is inherently related to health, but NYHIPA’s broad definition of “regulated health information” could sweep such information into its scope.

NYHIPA should be updated to narrow the definition of the term “regulated health information.” We recommend that the definition of “regulated health information” be aligned with the majority of states that have passed legislation regulating “consumer health data,” such as the definitions of the term in the Connecticut, Maryland, and Nevada laws. In those state laws, “consumer health data” is personal information that is linked or reasonably capable of being linked to a consumer and that “*is used to identify*” the past, present or future health status of the consumer. This definition will help ensure that the term is cabined to information actually used to identify a consumer’s health status. This ensures that less sensitive data points are not unintentionally swept into the definition of the term.

NYHIPA should also be amended to clarify that “regulated entity” refers exclusively to entities processing “regulated health information” of New York residents. We recommend refining the existing broad term defining a “regulated entity” to remove entities that control the processing of “regulated health information” about individuals “physically present” in New York.³ This update will help clarify that the bill applies exclusively to the “regulated health information” of New Yorkers and would help harmonize the bill’s terms with laws in other states.

II. The Bill’s “Valid Authorization” Requirements Would Cause Consumer Frustration Without Providing Meaningful Privacy Protections

In part due to the NYHIPA’s broad definitions, the requirement to obtain “valid authorization”—a signed consent—every time a regulated entity processes regulated health information would inundate New Yorkers with an overwhelming number of consent requests for even the least sensitive data processing activities.⁴ NYHIPA would permit a regulated entity to process regulated health information without a valid authorization for a narrow set of purposes, and would explicitly prohibit processing for “activities related to marketing, advertising, research and development, or providing products or services to third parties” absent valid authorization from the consumer.⁵ As a result, NYHIPA valid authorization requirements would significantly hinder the use of data to improve products or services, conduct research for the benefit of consumers, and apprise consumers of relevant offerings that may interest them – such as a coupon on shampoo – without obtaining signed consent for such activities. NYHIPA would also require consumer consent to be specific to the processing activity.⁶ *NYHIPA is consequently likely to result in significant consent fatigue for New Yorkers instead of providing meaningful privacy protections for consumers.* NYHIPA’s detrimental—and likely unintentional—consequences would hinder consumers from receiving significant benefits associated with routine and essential data practices

³ *Id.* at § 1120(4).

⁴ *Id.* at § 1122.

⁵ *Id.* at § 1122(1)(b)(ii)(B).

⁶ *Id.* at §§ 1122(2)(a)(i), (iv).

while simultaneously placing overly burdensome requirements on regulated entities that process any information that might even vaguely be related to health – or not.

III. Requiring Regulated Entities to Pass Consumer Rights Requests to Third Parties Could Conflict with Consumer Preferences

NYHIPA would require regulated entities to pass along deletion and correction requests to third parties and would require any third party that receives notice of such a request to delete all regulated health information associated with the consumer within 30 days of receipt of the request.⁷ NYHIPA should not require regulated entities to flow deletion requests through to third parties, because such actions may not align with consumers' wishes. A consumer may desire, for instance, to delete the regulated health information maintained by a certain regulated entity but may not want other regulated entities—who would be “third parties” under the NYHIPA's terms—to similarly be required to delete regulated health information. Such a requirement could impact products and services the consumer wishes to and expects to receive, as a deletion request served on one entity would be required to be cast to all other entities in the marketplace. The consumer may not intend to or want to serve a deletion request on regulated entities the individual knows, trusts, and wishes to continue to receive messaging from. Requiring deletion requests to be passed to other entities in the marketplace would create detrimental results the individual did not intend, desire, or expect.

IV. NYHIPA's Unlimited Attorney General Rulemaking Authority Would Increase Variation with Other State Privacy Laws.

As drafted, NYHIPA would give the New York Attorney General (“AG”) broad authority to “promulgate such rules and regulations as are necessary to effectuate and enforce the provisions of this section.”⁸ Such authority would exacerbate the inconsistency that already exists amongst state health privacy laws by enabling the AG to issue rules that are out of step with privacy requirements in other states. Harmonization with existing privacy laws is essential for creating an environment where consumers in New York and other states have a consistent set of expectations, while minimizing compliance costs for businesses. Several sections of NYHIPA should be updated to harmonize the bill with similar laws in other states. For instance, NYHIPA should be revised to define the term “sell” to exempt transfers to service providers, include appropriate exemptions to consumer rights and other obligations to ensure “regulated health information” may be maintained to prevent and detect fraud and respond to security incidents, and amend the data minimization provisions by changing “strictly necessary” to “reasonably necessary,” consistent with the approaches taken by other state laws.⁹

Compliance costs associated with divergent privacy laws are also significant. To make the point: a regulatory impact assessment of the California Consumer Privacy Act of 2018 (“CCPA”) concluded that the initial compliance costs to California firms for the CCPA *alone* would be \$55

⁷ *Id.* at §§ 1123(2)(c), (d).

⁸ *Id.* at § 1127(6).

⁹ *Id.* at §§ 1120(5), (2); § 1122(1)(b). *See* Wash. Rev. Code § 19.373.010(26)(b)(ii); Conn. Gen. Stat. § 42-524 (f).

billion.¹⁰ Additionally, a recent study on a proposed privacy bill in a different state found that the proposal would have generated a direct initial compliance cost of between \$6.2 billion to \$21 billion, and an ongoing annual compliance cost of between \$4.6 billion to \$12.7 billion for companies.¹¹ Other studies confirm the staggering costs associated with different state privacy standards. One report found that state privacy laws could impose out-of-state costs of between \$98 billion and \$112 billion annually, with costs exceeding \$1 trillion dollars over a 10-year period and small businesses shouldering a significant portion of the compliance cost burden.¹² New York should not enact a law that could add to this compliance burden for businesses. The legislature should remove NYHIPA's rulemaking authority, or at the very least take steps to place more limitations on the rulemaking authority to foster more consistency across state regimes.

V. Clarify the Bill Does Not Include a Private Right of Action.

To ensure that enforcement remains within the Attorney General's purview and to avoid any confusion regarding who can take legal action, NYHIPA should explicitly state in the bill that it does not grant a private right of action. Without an explicit statement, potential legal ambiguity could create confusion among businesses and consumers.

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¹⁰ See State of California Department of Justice Office of the Attorney General, *Standardized Regulatory Impact Assessment: California Consumer Privacy Act of 2018 Regulations* at 11 (Aug. 2019), located at <https://www.oag.ca.gov/sites/all/files/agweb/pdfs/privacy/ccpa-isor-appendices.pdf>.

¹¹ See Florida Tax Watch, *Who Knows What? An Independent Analysis of the Potential Effects of Consumer Data Privacy Legislation in Florida* at 2 (Oct. 2021), located at <https://floridataxwatch.org/DesktopModules/EasyDNNNews/DocumentDownload.ashx?portalid=210&moduleid=34407&articleid=19090&documentid=986>.

¹² Daniel Castro, Luke Dascoli, and Gillian Diebold, *The Looming Cost of a Patchwork of State Privacy Laws* (Jan. 24, 2022), located at <https://itif.org/publications/2022/01/24/looming-cost-patchwork-state-privacy-laws> (finding that small businesses would bear approximately \$20-23 billion of the out-of-state cost burden associated with state privacy law compliance annually).



We and our members support protecting consumer privacy. We believe, however, that NYHIPA takes an overly broad approach to the collection, use, and disclosure of any data that could possibly be related to or indicative of health. We therefore respectfully ask that you carefully consider these negative impacts of the NYHIPA and veto to the bill. If a veto is not possible, we would be happy to work with you on amendments to the bill to incorporate the points discussed in this letter.

Thank you in advance for your consideration of this letter.

Sincerely,

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